



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : هبه احمد جمعه كامل
المحافظة : القاهرة
الحى / المدينة : البساتين
رقم البطاقة : 28312180103926 رقم التليفون : 01116204391
العنوان : //
العمر (تاريخ الميلاد) : 18-12-1983 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 27-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام رنين مغناطيسي علي المخ والحجاج بالصبغه
قسم العلاج :
الأورام

التعليق على التشخيص :
[1112]

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: هبة احمد جمعه كاما National ID: 28312180103926	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حمید بالثدى و تحتاج الى Contrast-enhanced MRI of the brain and orbits
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



Name: هبه احمد جمعه كامل MR. : 7336			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 42-year-old individual who underwent upfront surgery in February 2023 with pathological stage PT2 M0 M0. Post-operative management included adjuvant chemotherapy, radiation therapy, and an 8-month course of adjuvant tamoxifen. The patient subsequently experienced vaginal bleeding, prompting dilation and curettage, which returned negative for residual disease. Since June 2024 the patient has been maintained on aromatase inhibitor therapy with Arimidex, now for 1.5 years. In January 2026 the patient received a dose of Zometa in conjunction with Arimidex and a three-month supply of supportive medication, with the next scheduled Zoladex dose on 4 June 2026. The patient presented for continuation of Arimidex and Zoladex supportive therapy, noting a three-month supply of supportive medication as of July 2025 and confirming that the regimen remained free of issues. The patient was seen for follow-up of Arimidex plus supportive therapy, with a three-month supply of supportive medication documented as of May 2025. The patient was noted to be free of complications and was scheduled to receive Zoladex and Zometa from the pharmacy on 28 June 2026. The patient was evaluated in the medical oncology clinic for ongoing Arimidex, Zometa, and supportive therapy. Laboratory studies performed on 6 June 2026 were reviewed and accepted. The current plan is to continue the existing endocrine and bone-protective regimen pending further assessment.			
تحريرا في ,			
التاريخ :		18-09-2026	
الوقت :		هنا اسامه عبد الرحيم عبد البر	
الطبيب المعالج :		يعتمد ,	

Request No. : 13004943
Request Date : 25/02/2023
Verification Date : 16/03/2023
Financial Category : *المرضى من دار السلام - نفقة الدولة*
Department : Out Patient
Registered by : marwa mohamed young

Author's Address:

Prof. Gamal Abdelatty
Dr. Israa Elgendy

Dr.Marwa mahmoud

Page 2 of 2

Printing By : marwa mohamed you

Patient Name : الأستاذة / هبة احمد جمعه كامل	Request No. : 13002467
Patient No. : 28312180103926	Request Date : 21/04/2024
Path No. : 1308/24	Verification Date : 28/04/2024
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 40 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Abnormal uterine bleeding.

Nature of Specimen: Dilation & Curettage, collected at 24/04/2024.

Gross Examination.

Multiple grey tan tissue pieces, collectively measured 1.5x1.5 cm, totally submitted.

Microscopic Examination:

Histopathological examination revealed few fragments of endometrial tissue with normal glandular / stromal ratio, formed of tubular & irregularly dilated glands, lined by pseudostratified columnar epithelium among spindle cell stroma. No specific inflammation, atypia. No malignancy in section examined.

Diagnosis:

Endometrium, Dilation & Curettage:

Disordered proliferative endometrium, No malignancy.

Signature

Prof. Dr. Gamal Abd-El-Atty.

Dr. Esraa Magdy.

Verified by :

Dr. Duaa Mohamed

Printing Date : 04/05/2024 08:40:42

Page 1 of 1

Printing By : marwa mohamed youss



شرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذة / هبة احمد جمعه كامل	Request No.	: 13000599
Patient No.	: 283125180103926	Request Date	: 31/12/2022
Path No.	: 3916/22	Verification Date	: 09/01/2023
Sex	: Female	Financial Category	: اورالم دار السلام - نفقة الدولة
Age	: 39 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
 Right breast mass lesion
 Nature of the specimen:
 Tru cut biopsy collected 29-12-2022 (1:42PM)
Gross Examination:
 Multiple greyish white and brownish tissue cores up to 1.5 cm. Totally submitted.
 Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:
 Histological type: Invasive duct carcinoma with focal lobular carcinoma
 Grade (differentiation+ nuclear pleomorphism +, mitosis): I (3+2+1)
 Angiolymphatic invasion: No
 Local insitu component
 Additional pathologic findings: Microcalcification
IHC:
 ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
 PR: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
 HER-2: Weak incomplete membrane reaction in tumor cells (score 1+)
 Ki67: 10 % nuclear reaction in tumor cells

Diagnosis:
 Right breast mass lesion, Tru cut biopsy :
 Invasive duct carcinoma with focal lobular carcinoma grade II
 Local insitu component
 ER: Positive (8/8)
 PR: Positive (8/8)

Verified by :
 dr.esraa magdy

Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	الأستاذة / هبة احمد جمعه كامل	Request No.	13000599
Patient No.	283125180103926	Request Date	31/12/2022
Path No.	3916/22	Verification Date	09/01/2023
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	39 Y	Department	Out Patient
Referring Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

HER2: Negative (score1+)
Ki 67: 10 % nuclear reaction in tumor cells

Prof.Dr.Manal Elmahdy
Dr.Israa Elgendy

Verified by :
dr.esraa magdy

Printing Date : 19/01/2023 10:21:59

Page 2 of 2

Printed By : .marwa mohamed yousse

Patient Name: Heba Ahmed Gomaa (Camel)	Date : 21/07/2025
Exam : MAMMOGRAPHY BOTH	Age : 41 Year(s)
Medical ID:	Pat. ID: 27684

FOLLOW UP BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

INDICATION:

Right CBS

TECHNIQUE:

- Bilateral MLO and CC views
- Bilateral breast ultrasound

REPORT:

- Fairly dense fibroglandular densities, (ACR A), of both breasts.

RIGHT BREAST:

- Focal area of parenchymal distortion is still seen underlying the scar site with a retroareolar well defined lesion, and skin and trabecular thickening, all owing to the prior surgical interference and radiotherapy.
- Complementary U/S showed the underlying large cystic lesion with thickened walls and soft tissue component, measuring about 3.8x2.7cm. Colour Doppler depicted no abnormal flow.
- No definite pathological axillary node.

LEFT BREAST:

- A small well defined rounded lesion is seen in the lower midline. complementary ultrasound depicted an underlying cyst, measuring 4.6mm.
- Normal skin thickness and nipple-areola complex.
- Non-specific axillary nodes.

OPINION:

- Right breast post operative sequel, with fat necrosis. (BIRADS 2)
- Left small cyst. (BIRADS 2).

Best regards,
Dr. Mahi Mokhtar

Ministry Of Health
Specialized Medical Centers
Dai El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
مركز الأورام التخصصي
مستشفى داء السلام
إدارة الأشعة

Patient Name: Heba Ahmed Gomaa Kamel	Date: 16/01/2024
Exam: MAMMOGRAPHY BOTH	Age: 40 Year(s)
Medical ID: 220/2023	Pat. ID: 27684

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Right breast conserving therapy

Report:

- Scattered fibro glandular density (ACR B)

Right breast:

- Operative bed parenchymal distortion and scarring showing a large retro-areola hypodense lesion seen by ultrasound as a complex cyst showing a soft tissue component measuring about 35x25mm. Picture is impressive of a large oil cyst with fat necrosis.
- No sizable mass
- No suspicious mass lesions, micro calcifications
- Diffuse edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Left Breast:

- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post right BCS status showing large oil cyst with no evidence of recurrence (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Best regards,

Dr. Eman Abdel-Razek MSc, FEBR

Dr. Amira H Radwan MD,

Breast Imaging consultant,

NCI, Cairo University

16/01/2024 3:09 PM



Presidential Program for Women's health

Name: Heba Ahmed Gomaa Kamel

Date: 29/12/2022

Bilateral Full field Digital Mammography

Clinical summary: 39 y old female patient coming with right breast palpable lump for about 2 weeks.

Technique

Mediolateral oblique and CC views

High resolution ultrasound

Detailed Findings

Left breast

- Scattered fibroglandular pattern (ACR B).
- No spiculations, masses, asymmetry, distortion or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic masses.
- Normal skin/nipple/areolar complex.
- Preserved axillary LNs.

Right breast

- Scattered fibroglandular pattern (ACR B).
- UOQ high density area of distortion is seen with anterior (towards nipple) and posterior speculations. No suspicious microcalcifications.
- Ultrasound demonstrated an area of altered parenchymal echogenicity mainly hypoechoic with irregular speculated hypoechoic solid mass (=23x12mm) within the same area and evidence of extension towards the nipple. The whole area is seen extending from 10 to 12 o'clock position measuring about 4x 2 cm.
- Normal skin/nipple/areolar complex.
- Preserved axillary LNs.

Conclusion:

Left breast: (BIRADS 1) Normal study.

Right breast: (BIRADS 4c) UOQ area of altered parenchymal echogenicity with malignant looking mass.

Recommendation: Biopsy was taken at the same settings.

Dr.Nahid Yahya,FEBR



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الست المصرية
صحة
مصر

MDT FORM

26/3/2023

G

Name	فهد محمد	MRN	220
Age	39	ID	2023
Menopausal status	pre-menopausal	Phone	01127197701
NRS SCORE		Marital status	M.
BMI		Family history	irrelevant
CO morbidity	irrelevant	REASON OF MDT	Post-operative

Clinical Data

BX:

ER:

2/8

PR:

2/8

HER 2:

-ve

Ki67:

10%

Sonomammography:

Mets Workup:

Post-operative (BS + SNL B)
→ Mixed invasive duct and lobular Carcinoma
→ Ductal Carcinoma in situ → Focal

TNM

Recommendations:

→ LN 0/8
→ Resection margins are free, Deep margin showed usual ductal hyperplasia

MDT OUTCOME & PLAN

- Port insertion
- TC x4 cycles
- PORT, adjuv. Tam thereafter switch to AI

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

Nutrition

Psychiatric

د. ياسر يوسف
د. نزيه

Wish

Patient Name: Heba Ahmed Gomaa Kamel	Date : 04/03/2024
Exam : ultrasound ABDOMEN& PELVIS	Age : 40 Year(s)
Medical ID: 220/2023	Pat. ID: 27684

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Average in size displaying homogenous parenchymal echopattern with smooth surface.
- No definite focal lesions or intra hepatic billiary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Surgically removed
- CBD is not dilated.

Spleen: -

average-size, homogenous.

Kidneys: -

- Both are of normal size, shape, echogenicity and well differentiated with good corticomedullary differentiation.
- No stones or dilated pelvi-calyceal system
- No gross masses or space occupying lesions could be seen.

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites.

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adenexa: TVUS was done

Opinion:

Cholecystectomy

Signature Dr . Nermien Ramadan

04/03/2024 2:12 PM